

Park Medi World (PARKHOSPS)

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BSE Limited
P.J. Tower,
Dalal Street, Fort,
Mumbai - 400 001
Scrip Code: 544645
National Stock Exchange of India Limited
Exchange Plaza,
Bandra -Kurla Complex, Bandra (E),
Mumbai - 400 051
SYMBOL: PARKHOSPS

Subject: Disclosure under Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("Listing Regulations") - Transcript of Earnings Conference Call on January 29, 2026

Dear Sir/Madam,

Pursuant to the provisions of Regulation 30 of Listing Regulations, please find enclosed transcript of the Earnings Conference Call held on January 29, 2026 for Unaudited Standalone and Consolidated Financial Results for the quarter and nine months ended December 31, 2025 .

The transcript is also being disseminated on the Company's website at <https://www.parkhospital.in/>

This is for your information and records.

Thanking you,

For and on behalf of Park Medi World Limited

Name: Abhishek Kapoor
Designation: Company Secretary & Compliance Officer

Encl: A/a

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"Park Medi World Limited
Q3 and nine -month FY '26 Earnings Call "
January 29, 2026

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MANAGEMENT : DR. AJIT GUPTA – CHAIRMAN – PARK MEDI WORLD LIMITED
DR. SANJAY SHARMA – WHOLE TIME DIRECTOR AND CHIEF EXECUTIVE OFFICER – PARK MEDI WORLD LIMITED
MR. RAJESH SHARMA – CHIEF FINANCIAL OFFICER – PARK MEDI WORLD LIMITED
MR. SUDESH SHARMA– CHIEF STRATEGY OFFICER AND OSD FINANCE – PARK MEDI WORLD LIMITED

MODERATOR : MS. SALONI NAGVEKAR – ADFACTORS PR

Moderator: Ladies and gentlemen, good day and welcome to the Park Medi World Limited Conference Call hosted by Ad factors PR. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the call, please signal an operator by pressing star then zero on your touchtone phone.

I now hand the conference over to Ms. Saloni Nagvekar from Ad factors PR. Thank you and over to you, ma'am.

Saloni Nagvekar: Thank you, R ayo. Good afternoon, everyone, and welcome to the Q3 and nine -month FY '26 maiden earnings call of Park Medi World Limited . Today, we have with us Dr. Ajit Gupta, Chairman, Dr. Sanjay Sharma, Whole Time Director and CEO, Mr. Rajesh Sharma, Chief Financial Officer, Mr. Sudesh Sharma, Chief Strategy Officer and OSD Finance, and Ad factors IR team .

We will begin the call with the opening remarks from the management, after which we will have the forum open for the interactive Q&A session. I must remind you that this conference may include forward looking statements about the company, which are based on the beliefs, opinions, and expectations of the company, as on date of this call. The statements are not the guarantee of future performance, and involve risks and uncertainties that are difficult to predict .

I now hand the conference over to Dr. Ajit Gupta, Chairman of Park Medi World Limited , for opening remarks. Thank you and over to you, sir.

Ajit Gupta: Yes. Good afternoon, everyone, and a warm welcome to Park Medi World Limited Earnings Conference Call. Thank you for joining us today, and for taking out the time to engage with us. This call is particularly meaningful for us, as it marks Park Me di World's first ever Earnings Conference Call.

While our hospital has been serving the patient for the last two decades, today marks the beginning of a more structured and transparent dialogue with our investor community. We look forward to building a long -term relationship with all of our stakeholders , through consistent communication and clarity of strategies.

Park Me di World was founded on the belief that has remained unchanged since the day, and that the quality healthcare should be accessible, affordable, and dependable, irrespective of the geography or income level of the patient. Our journey began in 2005, when we established Park Hospital in New Delhi. At the time, the focus was not on the scale, but trust, delivering ethical medical care, strong clinical outcomes, and patient -first decision -making.

Over the years, that foundation has enabled us to grow steadily, without compromising on our core values. Today, Park Hospital stands as the largest private hospital chain in Haryana, and North India's second -largest private chain hospital. We currently op erate 14 multi -super
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specialty hospitals, with an installed capacity of 3,250 beds, including approximately 870 ICU beds across Haryana, Punjab, Delhi, and Rajasthan.

Our growth has been driven by a cluster -based expansion strategy, which we believe is one of our strongest competitive advantages. Instead of spreading over thin geographies, we focus on building dense regional clusters. This approach allows us to scale re sponsibility while maintaining consistency in clinical quality and patient experience.

Another key pillar of Park Medi World or Park Hospital is our doctor -led professional management module. Clinical leadership plays an active role in operational decision -making, ensuring that growth initiatives are aligned with the patient outcomes. Across our network, we offer 30 super specialties and specialty services, supported by a strong team of more than 1,200 doctors and more than 2,400 nurses.

We have consistentl

y invested in advanced medical technology, including robotic -assisted surgeries, modern diagnostic and critical care infrastructure. These investments help improve clinical precision, reduce recovery time, and enhance patient experience, while remaining aligned with our affordability -focused model. Inorganic growth has been another important drive of the scale.

Over the years, we have demonstrated a strong ability to acquire and integrate hospitals successfully, applying a standardized operating playbook. Recent acquisitions such as KPS Institute of Medical Sciences , Agra and Krishna Super Special ity Hospital , Bathinda further strengthen our presence in the high -potential region and fit well within our cluster strategies. As we look ahead, our vision is to continue building a scalable, sustainable healthcare platform

focused on expanding capacity in an unpenetrated market, strengthening specialty depth, maintaining affordability and quality, creating long-term value for the stakeholders. With that, I would like to hand over to the Whole Time Director and the CEO, Dr. Sanjay Sharma, who will take you through our operations and executions. Over to you, Dr. Sanjay.

Sanjay Sharma: Thank you very much, sir. Good afternoon to all the invitees. Firstly, heartfelt gratitude to all the invitees from Park Medi World Family for taking this initiative of joining us and showing keen interest in our organization.

As you all would be aware, we have come out with our Q3 results. In the healthcare delivery parlance, this is the quarter where it is supposed to be a healthy period, and generally the results are subdued. But I'm glad to inform that we have gone ahead and we are still on track and ahead of the curve, which my financial director, CFO, will be telling about it.

Largely why we have been able to attain these good results is based on four pillars of core value. Over and above that, Park Medi World's strength, as our Chairman mentioned, is patient-centric care and patient satisfaction. And they are followed by these four pillars of core value. One, first

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one, is the highest quality. All our hospitals are NABH accredited and major labs are NABL. We constantly endeavor in improving our operational efficiencies. We constantly are on the improvement towards our clinical outcomes.

The second one is affordability. Largely it starts with our being not centric-based because these are large states with the larger population and the gap between the population and bed is huge. And also the disease prevalence is very high. The concept of 250-bedded hospital gives us high operational efficiency and quick break-even. And capex, around INR34 lakhs, is also one of the main factors which keeps our affordability aspect.

The bed configuration of 30% being dedicated to critical care and 40% to general ward is another factor which contributes towards affordability. The other factor is full-time doctors. And as Chairman mentioned, the cluster growth, this gives us great synergy, multiple optimal utilization of HR, resources and equipments.

The third one is higher technological adoption and training. That we have been doing by increasing our HIS standardization, clinical protocols, more onto data analytics, expanding advanced medical equipments like robotics. We are one of the groups in best of our knowledge which has obtained three da Vinci fifth-generation robots. And we've been conducting heart surgery through robots, joint replacement robot-assisted organ transplants, neurointervention, cardio interventions and others.

Training and the clinician aspect of it is largely based on that people remain our focus area. And training and clinical governance and leadership development is our main focus. This is also seen from this point of view that seeing our academic strength and deep, you could say A to Z, the

patient diseases, we have been accorded and awarded 63 DNB seats by the government in specialties and super specialties. These initiatives support better clinical outcomes, faster recovery and improved patient satisfaction when we come across into the product mix.

The fourth and the most important consistent growth and profitability. This has come largely, regularly from past 21 years. And in future also we intend showing it. And this is because we are going for higher and higher operational efficiency, ramping up our occupancy. And despite the new addition of B athinda of 250 bed, our occupancy has risen to 65%.

And higher tertiary and quaternary product mix, wherever the occupancy is higher, we go for high end quaternary and product mix. As I mentioned to you, the robotic joint replacement, robotic organ transplant, neuro intervention, cardiac intervention, which has led to increase in our ARP OB from 25,500 to 27,500 approximately, and which is decreasing ALOS to close to six days.

Also, the acquisitions of Greenfield and Brownfield is another aspect through which we achieve the growth. If I give you some future projections, in February, we are starting a project in Agra, KPIMS, which is a 360 bedded hospital. And in March first week, we will be starting a

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Greenfield project in Panchkula, which is 300 bedded project. So in FY '26 itself, from 3,250 we will be adding 660 beds more, which will take us to approximately 3,910 beds.

In FY '27, we are adding 500 more beds, 300 in Kanpur and in Delhi 200. And in FY '28, we will be aspiring for 850 beds, Gorakhpur 400, Amba la Extension, Onco dedicated facility, 200 beds, and Rohtak Greenfield, 250 beds, which should take us to roughly 5,260 beds. While

maintaining the EBITDA of 27%, PAT of 17%, and annualized RO CE of about 21%, and ROE of 23%. And this is largely being achieved, because we have constantly been following our core values and strength of patient centric care.

Thank you so much. And now I'll invite our CFO, Mr. Rajesh Sharma, to continue and take it forward, please. Thank you.

Rajesh Sharma: Yes, thank you. Thank you, Dr. Sharma. And good afternoon, everyone. During Q3 FY '26, Park Medi World reported revenue from operation of INR4,100 million, representing year -on-year growth of approximately 18%. For nine months FY '26, revenue stood at INR12,189 million, reflecting growth of around 17% year -on-year, driven by steady patient volumes and the ramp up of newer and acquired hospitals. EBITDA excludes other income of Q3 FY '26, stood at INR994 million, with an EBITDA margin of 24%.

For nine months FY '26, EBITDA was INR3,170 million with a margin of 26%, reflecting stable operating performance amid ongoing integration of recently acquired hospitals. Profit after tax for Q3 FY '26 stood at INR528 million, while PA T for nine months FY '26 was at INR1,968 million, supported by operating leverage and disciplined cost management. Operational matrix during the period reflected disciplined execution.

Average occupancy across the network improved to approximately 65% during nine months FY '26. ARPO B increased to 27,406 and ALOS remained stable at around 6.34 days, indicating a balanced case mix and efficient clinical processes. Our mature hospitals continued to demonstrate operating leverage while newer and recently acquired hospitals progressed as per their planned ramp -up trajectory.

The strength of our mostly owned hospital asset base provides financial flexibility and supports long-term value creation. We remain focused on maintaining balance sheet strength while funding growth initiatives. Recent acquisitions are being integrated as planned, with emphasis on improving utilization, specialty depth, and cost efficiency.

Incremental capex requirements are expected to be largely optimization -led. Our capital alloca

tion priorities remain clearly defined, one, strengthening the balance sheet, funding organic and inorganic expansion, improving returns ratio over the medium terms.

Besides, you know that I also wish to share our comparative for nine months for FY '26 versus FY '25. In nine months FY '26 we achieved a revenue of INR12,189 million as compared to INR10,397 last year. So there is a growth of 17% in revenue. EBITDA also grown from Park Medi World Limited

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INR2,826 million to INR23,170. So there is a growth of 12% in EBITDA. And PAT is also grown from 1,374 million to 1,968 million and there is a growth of about 40% in PAT.

And I wish also like to mention about the footfall. IPD and OPD together, it was 5.32 lakhs patient last year that grown to 6.6 lakhs in the first nine months of the current year and we registered a growth of 24% in footfall. And our bed also increased from 3,000 to 3,250. As Dr. Sanjay mentioned that we added 250 bed in the current year, Bathinda. And ARPOB also improved from 25,500 to 27,400.

ALOS, it was 6.59 that slightly came down to 6.34 and our occupancy also grown from 62% to 65% despite we added 250 bedded hospital in the current year. And our capex bed remained at 3.4 million. In the last, it was same in nine months last year and the nine months in the current year.

With that, we conclude our speech. Thank you for your time and interest in Park Medi World.

We will now happy to take your questions. Thank you very much.

Moderator: Thank you. We will now begin the question and answer session. The first question is from Nirali Shah from Ashika Stock Services. Please go ahead.

Nirali Shah: Hi, I have three questions. First one is, can you share how the recent Agra acquisition fits into the broader cluster -based expansion strategy that we hold?

Sanjay Sharma: Sorry, could you repeat the question, please?

Nirali Shah: Yes, sure. I was just asking that the recent Agra acquisition that we have done, how does it fit well into the broader cluster -based expansion strategy that we hold?

Sanjay Sharma: See, we are looking Uttar Pradesh as a very big territory. It has got roughly 26 -27 crore of population, which is nearly twice the population of Russia, a little less than US from that point. And we see that the growth potential is huge. And the demand and the gap which is there in health services is also very vast. And this profile is also constantly growing.

So, we intend coming up in UP in a big way. This unit which we have taken up in Gorakhpur covers, this Agra covers one side of it. The other two, central and the other side is Gorakhpur and Kanpur. And in between, we intend building up cluster growth in Pe arl and Necklace to ensure that the UP is covered well. I hope I have been able to answer your question.

Nirali Shah: Yes, sir, understood. My next question is, as the new hospitals mature, what kind of margin

trajectory should we expect at the ground level, group level?

Management: Nirali, we believe that we will be able to sustain margins more or less in line with the margins we have right now. So, our EBITDA is expected to remain in 26 %-27% range going forward for mid to long term. I am seeing a time horizon of three , five, 10 years. And our PAT should remain consistent or stable. And I should say strong in 15%-17% range.

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Rajesh Sharma: I wish to add one more thing. Historically, wherever we made the acquisition, so what we have seen that whenever we achieved the occupancy of 30%, we achieved our EBITDA breakeven. And we have seen next year, maybe next year of the operation, we normally touch EBITDA margin of 20%. And third year, we remain come on the same margin that is 26 %-27%. So, historically, we have seen . In the Agra also, we are very, very confident to achieve those numbers.

Management: Also not

eworthy, our ROE and ROCE numbers are strong in 21% range, might increase a little going forward. ROE and ROCE has obviously converged a little because right now we are carrying a very, very minuscule loan on the books. So, ROE, ROCE number nearly are converging. But a strong stable and might see a slight curve going forward.

Nirali Shah: So, sir, on a three to five year perspective, we are expecting the margin to remain stable or any basis points growth are we expecting?

Sanjay Sharma: See, Nirali, as per growth, as I mentioned that in FY '28, the bed strength will be growing up. We are aspiring for about 5,500 to 6,000. But in the pipeline, we have 210 beds. After that, another five years, we are looking at doubling this capacity to about 10,000. And since the past 21 years, we have been giving this EBITDA of 27% and PAT of 17%. And in coming years, also 10 to 15, 20 years, we expect that our margins will be around this value only.

Moderator: Thank you. The next question is from Tanay Bheda from Kotak Mutual Fund. Please go ahead.

Tanay Bheda: Hi, good afternoon. Congratulations on the IPO and the strong set of quarterly results. I have two questions. The first question is on disallowed claims. I want to understand how are we managing disallowed claims? What systems do we have? And how do we see this trajectory going forward? What differentiates us with sort of minimizing disallowed claims going forward?

Sanjay Sharma: Thank you, Tanay. This is Dr. Sanjay Sharma. The IPO success and the Q3 results are all because of your support and blessings and organizations like yours. The disallowed claims, what we have done is this, that we have a strong clinical team of super specialists which we have created. And largely, this is a difference of opinion between the organization which is making the claims and the organization which has submitted the claims.

And it is based on the clinical inference of the data which has been put in the discharge sheet. Now, this clinical team or the team of super specialists has been able to clarify most of the queries. And we have brought down the disallowance to about 8% to 9%, which is the lowest in the industry. Even in TPA insurance and cash, this disallowance is ranging anywhere between 12% to 15%.

So currently, our disallowance is at the lowest level of 8% to 9%. And we intend continuing at this level. Below that, this is the integral part of the healthcare delivery. This much disallowance would be there in any healthcare industry delivery model.

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Tanay Bheda: Okay. Okay. And my second question is just a follow-up on margins. So very soon, we would be seeing the benefit of CGHS rate increases . And with regards to this benefit, how much incremental revenue can be booked because of this CGHS rate increase? And how much of that can translate into EBITDA margin increase when we see the full year of benefits?

Management: Dr. Sanjay, for you?

Sanjay Sharma: Okay. See, the rate hike this time which has been done by CGHS has been one of the most handsome hike which has been done. And incidentally, since our government insurance, you could say the percentage is around 80%. So we are the maximum beneficiary in this. The hike has been around 12% to 15%. But this will take a little time because this hike will be all around from CGHS will percolate to ECHS, railways, National Highway Authority of India, the paramilitary, the police force, the state governments, electricity board, and others. So it takes a little time.

In the projections which we have given, we have not incorporated this hike currently. But from the new financial year, FY '27, we definitely see some part of this coming into our EBITDA.

But we will be conservatively accounting only 7.5%. Though the hike has been nearly close to

12% to 15%, but we have always believed in this talk less and

deliver more. So we will like to

pleasantly surprise our investors. Instead of giving higher figures, we like to give conservative figures and deliver more.

Tanay Bheda: Okay. Thank you. Thank you so much. That's it from us.

Moderator: Thank you. The next question is from Amit Agicha from HG Hawa and Company. Please go ahead.

Amit Agicha: Yes, good afternoon, sir. Thank you for the opportunity and congratulations for good set of numbers. So my question is connected to like, what is the ROI on robotics? Would you be able to share?

Sanjay Sharma: See, thank you, Amit, for the good wishes with regard to the result. To best of my knowledge, I have been into healthcare for about nearly four decades. And any organization taking three robotics, fifth generation da Vinci simultaneously, I'm not aware about it, at least in my personal knowledge. But we have taken them up. And the turnover, which we have been doing at an affordable cost, that has given us an excellent product mix.

Currently, we are doing kidney transplants, nearly about 25 to 30 per month. With robotics, we are also doing the robotic cardiac surgeries. We are also doing robotic assisted nearly 300 plus joint replacement. And what it has done is this, it has improved our product mix, which has led to faster patient recovery, lesser stay in the hospital, and also improved our ARPOB from 25,500 to roughly 27,500.

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We want to maintain our ARPOB in the affordable section only. So we are not looking at a huge increase in our ARPOB. But this definitely does improve the EBITDA margins, ensuring the affordability.

Amit Agicha: And sir, would you be able to put some light on, like what are the debt levels post the IPO?

Rajesh Sharma: So I think, you know, as far as debt is concerned, so pre-IPO, we used to have a total term debt of INR425 crores, of which we have already paid INR380 crores. And some of the repayment is done in last few months. So if I talk about as of 31st Jan, the total term debt will stand at close to INR15 crores, which we also plan to repay in Feb. So by end of Feb, we will be completely debt-free company.

Amit Agicha: And sir, are there any plans for any new verticals ...

Sanjay Sharma: Sorry, say again.

Amit Agicha: Any plans for entering new verticals, like for example, kidney transplant? I think so you're already doing, I think in five hospitals, right?

Sanjay Sharma: See, we are already doing. Robotics, we have entered. Organ transplant we have entered, I think, barring liver transplant and heart transplant, we are doing all high-end tertiary and quaternary medical procedures, which are being done anywhere in the world currently.

Amit Agicha: The last question from my side. Any wage inflation guidance?

Sanjay Sharma: Sorry, any?

Amit Agicha: Wage inflation guidance, like the employee cost.

Sanjay Sharma: I have not been able to understand employee cost.

Amit Agicha: Inflation guidance.

Sanjay Sharma: Inflation guidance.

Amit Agicha: Yes. With respect to employee cost.

Sanjay Sharma: Okay. So we have full-time doctors, Amit. And this is one of the models which we have been following in the affordable section. What we do is generally our selection of clinicians are this way that we take a clinician who has five to eight years of hands-on experience from a fantastic good academic college, which is largely a government college background and who has had keen exposure into the latest technologies, robotics, and latest medical fields.

And they have hands-on experience. What we do is this. We give them independent status. We give them a unit and they are able to do the procedures because they have reasonable experience

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with excellent academic skills. And we account for only two major aspects. One is the patient and the

other is attendance satisfaction and the other is clinical outcomes.

And based on that, there is a graded percentile system which is created from each unit. And any clinician which scores more than 75 percentile is every month is personally appreciated by the

promoters and rewarded a handsome performance bonus. So this is part of this which all these inflation which is coming into it is actually being supported by the patient flow and the volume, which in turn increases the revenue.

So both are going in a balanced formation. So we do not go for any celebrity doctor which costs very high and which leads to high inflation in the HR. In fact, our HR cost and the doctor's consultant's attrition is the least in the industry.

Amit Agicha: Sir, may I know the attrition rates at this point?

Sanjay Sharma: Sir, our attrition rate at the consultant level is about 18.9 %, which is the lowest in the industry.

Amit Agicha: I appreciate you answering all the questions with so much clarity, sir. Thank you and all the best for your future quarters.

Moderator: Thank you. The next question is from Mukesh from Aster Ventures. Please go ahead.

Mukesh: Sir, my first question is that we have not added any bed in Q2 to Q3. Our bed is similar, but our consultation fee for doctors and doctor's salary increased by 8 % on Q2Q. What is the reason of such increments, sir?

Management: Your voice is breaking.

Mukesh: I am saying, sir, our doctor fees and consultation fee increased by 8 % in QtoQ. What is the reason of such increase?

Rajesh Sharma: See, what we have done, as Dr. Sanjay also mentioned, we are just coming up with a unit in Agra. And also, we are planning to start a unit in Panchkula . So, there are a lot of hiring is happening.

Mukesh: Okay. There is a lot of hiring in...

Rajesh Sharma: Hiring is happening. And also, you know, that as I said, because last time also that we mentioned, wherever we reach occupancy of 75 %, we start focusing on advanced super speciality. So, we need more senior consultants , so, there also we invested money. So, basically, there are three reasons.

We are coming up in Agra, where the hiring is already started. Panchkula hiring is already started. We already have a few people on board, plus hiring of senior consultants focusing on advanced super speciality.

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Mukesh: Okay, but in that case, we have already paid the amount in December for the November month with respect to hiring people.

Rajesh Sharma: Yes, because you have to have, because, you know, when, when, because hiring cannot be done immediately. You have to have people in place and you have to train them. We already have people on board. They will take care of Agra and Panchkula unit.

Sudesh Sharma : Okay. We would like to state here that we are commissioning our Agra facility of 360 beds by 15th of February and Panchkula organic facility with a capacity of 300 beds by first week of March. So, 660 bed capacity will be added within this financial year. So, typically, we launch with all the specialists provisioned on day one. And this hiring usually, therefore, has to be done before the commissioning of the hospitals. And therefore, the manpower cost looks like that for quarter three.

Mukesh: Okay. Sir, my second question is that, in presentation, we have not given payer mix slide. So, roughly what percentage of...

Rajesh Sharma: So, I'll give the answer for this question. If I talk about as of 31st December, our payer mix, you know, that relate to government scheme that came down to 83% from 84%. And now, self -pay.

Self-pay is 9% and TPA is 8%. So, there also, you know, that from 84 %, we came down to 83 % in government panels.

Sanjay Sharma: Mukesh, I'd just like to add this. I'll just like to add in this 83%, about 93% is central government. So, that's like a sovereign government. There is no default i

n that.

Rajesh Sharma: And there is a drastic improvement in terms of, you know, the government, they are also pushing it very hard internally to improve the system. So, recovery is also getting improved.

Mukesh: Okay. Okay. Okay, sir. What is the going forward trajectory for CGHS scheme? It remains there or it's going down, going forward?

Sanjay Sharma: So, the Central Government Health Scheme and the Central Government Patient Beneficiary is constantly increasing, one. And also, the rate hike, generally, in the past track record, if we see, they have been hiking the rate s every two, two and a half years. So, this has been happening on a regular basis.

So, this time, the rate hike has been very, very handsome. But I believe going further also, as the healthcare requirements are constantly increasing, and the government allocation of the healthcare budget is also being proposed to be increased from 3.8% o f the GDP to 5.5% by 2030.

So, I'm sure the allocation for the Central Government Health Beneficiaries will also have a major contribution into it.

Mukesh: It means that the ratio is remaining here and there only?
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Sanjay Sharma: No, our focus, see, Mukesh, we are not going for any government insurance or government schemes. Our focus has been on the affordable side. So, we have been focusing on the bottom and the middle of the pyramid. Largely, the population of 128 crores out of 140 crores, which cannot afford healthcare. Now, incidentally, this segment is being largely supported by the government and in that too, also major aspect is coming from the central government. So, our focus will be in this segment only. But per se, the insurance penetration has been increasing. So, obviously, a lot of people from this segment will also translate into the private insurance. When we came for IPO, we started at around 84 %-16% distribution. Today, we are 83%-17%. I believe by the end of this financial year, we will probably be 80 %-20%. And going forward in a year's time, we will be 75 %-25%.

75% from the government insurance and 25% from the private insurance. So, our focus remains there. But a lot of patients are translating from this affordable section to the private insurance.

Mukesh: Thank you very much.

Moderator: Thank you. The next question is from Shreya Chatterjee from Ageless Capital. Please go ahead.

Shreya Chatterjee: Thank you, sir, for taking my questions and congratulations for a good set of numbers. My first question is on the payer mix itself. You have mentioned that government insurance will always be a majority of your payment. So, how does it impact your receivable cycle? Because I understand that it's a bit on the higher side versus your peers.

So, where do we see the receivable days going forward? What is current receivable days and where do we see it going forward?

Rajesh Sharma: As far as receivable, Dr. Sanjay, you want to answer it? Please go ahead. No, no.

Sanjay Sharma: Please go ahead, Rajesh.

Rajesh Sharma: As far as receivables are concerned, first very important part is because they are mostly 93% related to the central government. So, when we say central government, it's like a sovereign bond and we have never seen any of the money got better. So, we receive money. But historically, what we have seen that government, they have a very, very well -defined process to clear the payment.

So, there are two verticals. One is that to pass the bill and second is to release the payment. So, normally passing because when the patient gets discharged, we have to upload our bill on a third - party portal and it is absolutely faceless and transparent. So, this process takes about three months' time, wherein a couple of, normally back and forth happen when, that we have to immediately respond to their queries.

And this process takes about three months' time to s

ettle the bill. And thereafter, they take about

45 days' time to clear the payment. So, historically, we have seen we normally get our payment

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within four and a half months, five, ten days here and there. But in last couple of months, we have seen improvement. Even the government is also pushing it very hard. And what we are expecting by end of this financial year, I'm hoping this four and a half month TAT will come down to four. And maybe going forward, it will be close to three and a half months.

And I think the other important point to mention, somehow this becomes one of our strengths.

Because, we enjoy sort of a monopoly in this domain because the kind of ARPO Bs that we have, the receivable cycle that we have, it is practically impossible for a bigger player like Vedanta, Fortis to come down to our area. And it is practically impossible for a smaller player to compete with us. So, somehow this becomes one of our strengths.

Shreya Chatterjee: Got it, sir. And so, my second question would be, you have a very unique strategy of capex per bed, which is like very low compared to peers. So, going ahead with all the acquisitions that you do, so what is your strategy to select the hospitals and do you expect the capex per bed to remain at these low levels only? How do you think about that? If you could just give a bit more color to that.

Rajesh Sharma: Dr. Sanjay?

Sanjay Sharma: See, when we have been generally seeing the distress assets, largely if I talk about the brownfield acquisitions, these assets have been in severe distress. And we see a few things into it. One is the strategic location. How much is the distress that could give us compelling values and deep discounting into it? The kind of medical services which are available in that area. And fourthly, and the most importantly, the headroom to grow in that area.

Based on that, we do multiple, you could say, valuation of the property. And then we arrive at the buying price. And generally, because we pay upfront, we get a deep discounting. And that is why we have been able to maintain a very low capex .

Shreya Chatterjee: And so going forward, do you expect this level to continue or like the ROE, ROC levels will sustain going forward?

Rajesh Sharma: I'll answer this question. You know that, as Dr. Sanjay also mentioned, the other point is because when we built our hospital, we are here to provide the rest of the healthcare services to the affordable segment. So we keep a couple of things in mind. One is 40% of our beds, we set aside as a general bed. And we specialize in critical care.

So 30% bed, we set aside for ICU bed. And balance 30%, that is mainly 20 -22% is a twin sharing, wherein one room has to be shared by two patients and rest is single room. So this allows us to have more bed in less space. So this is one point.

Second, your question in terms of going forward. So we are adding six more hospitals by March 2028, which we have already disclosed in our R HP, that was part of our document with SEBI .

Wherein we are adding the first unit that we are adding in Agra, that will be up and running from Park Medi World Limited

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Feb 2026. Panchkula we are adding, that will start operation from March '26. Kanpur, next year we are expecting to add 500 beds, one in Kanpur and second in Delhi. And in FY '28, we are adding three hospitals, Gorakhpur, Ambala, Rohtak. Together we are adding 850 beds.

So we are adding 2,010 beds in next by March 2028, and on a capex of INR700 crores . So we will remain on our capex per bed of 35 lakhs. So we have done historically, we are currently doing it. And going forward also, we will remain on this model.

Shreya Chatterjee: Thank you, sir, for such a detailed answer. My last question would be more on the employment side, majorly related to doctors. So

you have so many specialized procedures. So what's the split between your specialized consultants versus full -time doctors, and what's your hiring and retention strategy, if you could just give a bit more color to it.

Sanjay Sharma: So Shreya, all our consultants are full -time. And if we employ only full -time doctors, there is no visiting consultant policy with us. And all these doctors which we hire, there is a future growth map which is mapped for them and told beforehand when the hiring is being done. There are a lot of things which we, from the HR point of view, also we ensure for their social well -being and settlement.

HR helps them in getting good accommodation. HR helps them in getting their children into good schools. These aspects we take care of to settle them. Within the system, we have this thing that these doctors are accounted not for volume or revenue at all. In fact, they're accounted only for two verticals largely. One is the patient and attendance satisfaction, and the other is clinical outcomes.

And based on that, they are rewarded based on a graded percentile system. And this reward is every month. And if anybody is scoring more than 90, 95 percentile, this reward can even go higher than their remuneration package per month. So there is a great satisfaction level at the consultant level.

Vice versa, the Park Hospital also gains mutually in return in a symbiotic relationship that the patient and attendance satisfaction is also very high . Because all the patients, especially the critical patients, are a number of times counseled in the trajectory of the treatment, in the prognosis, in the outcome. And all these aspects give them a lot of satisfaction because doctor gives them a lot of time.

They're not visiting , they are full time, so they're dedicated for this thing. So this is a major thing which is unique from the other hospitals. In the consultant level, our attrition rate has been the least in the industry, which is about 18.9 %. So most of the consultants which have been there with us, they have been there for 15 years, 12 years, 11 years, 10 years, so forth and so on.

Shreya Chatterjee: And so any specialized hiring strategy that you use to attract special talent for the consultants, like a special heart surgeon or someone from the oncology care? Is there any other special strategy that you follow?

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Sanjay Sharma: Yes, by the large bed strength and by virtue of being the second largest in North India and one of the listed companies, we by itself are a huge brand to reckon with. When we take out an advertisement for hiring, we get the cream of cream from the clinician point of view. If I give you a recent example, in Panchkula, when we started hiring for 300 bedded on Greenfield

project, which is upcoming, as Mr. Sudesh mentioned, in the first week of March, we got a footfall of about 4,500 to 5,000 people coming in.

And the best of clinicians in the Tri-City and around came to get hired with us. That is because they have been aware that how Park takes care of their clinician, not as an employee, but as partners, as part of the Park Medi World family. And they enjoy all the benefits in the growth, sustainability, profitability, and the growth prospects for future.

Management: When we look at hiring medical staff, it is four things combined that become, that have, if I can use the term sort of magnetic charm attracting, which enables us to attract high quality talent.

One is Park offers a very attractive remuneration policy, including both fixed and variable remuneration. Second is for incoming doctors, there is a chance to, opportunity to work on large number of cases, drawing on the Park's very specific strengths in the middle and bottom of the pyramid population segment.

Rajesh ji just now shared with you our IPD number in a nine month was nearly 75,0

00. And Dr.

Sanjay Sharma shared that we have 1,200 doctors of which 75% are specialists. Just imagine that number of specialists getting to work on that many number of patients. So large number of, opportunity to work on large number of patients. Third is an opportunity to assume significantly higher serious gravitas responsibilities early on. Should the doctors show that potential, that opportunity is there.

And fourthly, because some of our hospitals are located in tier 2 and tier 3 locations, I can safely tell you that Park has industry's most robust plan to assist the doctors. So should they have to relocate to any of these locations to assist them in adjusting to this location in terms of community support we provide, adjustment support we provide.

So combination of these four factors, attractive remuneration, opportunity to work on large number of patients, opportunity to assume higher responsibilities at an early age, and fourth is strong support to adjust to new locations. Should the doctor have to relocate? These four combination combined factors play a big role in attracting very high quality talent and requisite numbers as we require.

Shreya Chatterjee: Thank you, sir. Thank you for such a detailed response to all my questions.

Moderator: Thank you. The next question is from Saniket from Autus Wealth. Please go ahead.

Saniket: Thank you for this opportunity. We just wanted to know your business understanding. Like you have highlighted earlier in this call as well. A lot of our revenue basically comes from government schemes. Ultimately, our core focus is on the middle income and lower middle

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income, one-to-one debt because of patients in this country. But I wanted to understand how do we reduce our dependency and diversify our way? And sort of increase the revenue that's come from other sources of private insurance or direct walk in sort of patients. So could you please just highlight on how are we managing this?

Sanjay Sharma: Saniket, firstly, I would like to reaffirm and put our viewpoint again that our focus has always been in the affordable section. Whether they are being covered by any government insurance or they are self-payer from that point of view, either way, we will be focusing on the affordable section because that is a huge section. It is nearly 90%-92% of the population of India, and we have a huge headroom to grow in that.

We are not going for government insurance or government schemes per se. In this section, if anybody is being covered by the government insurance, they also come to us because the other premium segments are not entertaining them. We have a system in which we have already accounted in a rolling manner for this four and a half months of gestation period of the payment. In fact, as Mr. Rajesh Sharma mentioned, this becomes our moat for any new entrant to come in such large-scale structured format and to be catering to this affordable segment. So, we are unique in this manner that we are one of the largest players in this section, which is giving them high-quality affordable healthcare with the most, you could say, the best clinical outcomes.

Management: You know, think of it like this. We are as a business model not making a selection of patients in terms of their socioeconomic status or their location. Our business offers value proposition, affordable, high-quality services. So, we do not choose the patients on the basis of how they will pay for these services. Matter of fact remains that at the macro level, 90% of patients belong to bottom and middle of the pyramid, 10% to, if we can call it that, relatively affluent segment.

In Park, the mix of patients is similar. It is 83-17, 83% belonging to bottom and middle of the pyramid, 17% to relatively affluent, what is known as the cash TPA segment. So, business model is that regardless

of how patients will pay for them, it is not oriented at payment mechanism, it is oriented at patients.

And I think you will agree that for longevity of business, for long-term health of business, it is really very good if your patient mix is somewhat reflecting the reality that exists at macro level. If the patients, 83% patients who are from bottom and middle of the pyramid, should they want to pay us by means of cash and TPA, they are most welcome to do that. It is really coincidental that these patients covered by government insurance scheme and that becomes the payment mechanism. But we, per se, in our business model do not make selection of payments by distorting our business model, choosing patients belonging to certain socio-economic status. That is the answer to your question.

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Saniket: Right. And, sir, we are thankful for your explanation. Last one question from our end. We are incurring around INR700 crores of capex, right, for FY '28. So, how much are you incurring for FY '27? Could you, like, give us a timeline of this distribution?

Rajesh Sharma: So, as far as you know, the capex is concerned, out of the 700. So, this year, even the hospital which is coming up by next, before 31st March 2026, we are adding two units. One is Agra, second is Panchkula. So, they are together, because it is Agra that we are requiring, so wherein we are paying INR245 crores. And Panchkula, it was a greenfield where the construction is going on for last two years.

So, in the additional amount that we have to incur is about INR35 crores. So, together, you can say about 280 crores will go in the current year. Next year, FY27, we are adding 500 beds, wherein Kanpur, we have to invest about INR30 crores, and Delhi is about INR80 crores. So, only INR110 crores next year in FY '27.

Moderator: Thank you. The next question is from Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: Hi, sir. Thanks for taking my question. Sir, two things. One is, A, on the CGHS rate hike, which you said will progressively get rolled out, by what time do you think the entire impact will be visible for us? How much time will it take?

Sanjay Sharma: Hi, Nitin. This is Dr. Sharma. I think the effectivity has started percolating, but in totality, if we see all the organizations being able to effectively implement it, it takes about nearly three to six months. So, we are hoping by next financial year, the actual total implication of the rate hike will be coming into our finances.

Nitin Agarwal: Sir, is it safe to assume, by the second half next year, we should see the full impact of these rate hikes begin to reflect in the numbers?

Sanjay Sharma: Nitin, hopefully before that, but it will be safe to say by the second half of the next year.

Nitin Agarwal: And, sir, on an average, if there is a way to sort of classify it like that, what is the proportion of revenue hike that you have got on an average, on a weighted average basis, through these hikes?

If you were making INR100 earlier on government schemes, what is the weighted average increase that you have got through this scheme, through this rate hike that just happened?

Sanjay Sharma: Nitin, just to let you know, there are about 4,500 to 5,000 line items in the central government rate hike. So, for each line item and their implication on the finances will be difficult to predict prematurely. But generally, the overall rate hike, what we envisage has been about 12% to 15%. And the effectivity of that will come probably in the, as you said, second half of the next financial year.

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And also, what we see is this, that we will be still conservatively accounting for it. We will not be going for 12% to 15% surge the entirety into it. We will be looking at about 7.5% increment in our revenue

and EBITDA. That is conservative figure.

Nitin Agarwal: Got it. And, sir, on the UP that you mentioned is going to be a key focus area for you going forward. So, there is a hospital in Kanpur, Gorakhpur and Agra. Which are the other sort of areas that you would typically be looking for from an entry perspective as we go forward strategically?

Management: You know, it's a very, very interesting question. You would have noticed that one strategic plan of Park, whichever state we get into, is to reach a critical size. We have a critical size. We are largest in Haryana. We will have two units in Delhi now. We have four units in Patiala. We have two in Rajasthan. In Punjab, including Tri-City, we will have four units in Bathinda, Patiala. We have one in Mohali, one launching in Panchkula.

So, as we enter the state of UP, we always had a plan to launch a unit in Gorakhpur, in FY '27 and Kanpur in FY '27, Kanpur and FY '28 Gorakhpur. So, we had an opportunity and under very proficient set of circumstances to acquire a unit in Agra 360 bed capacity. So, if you visualize the map of UP now, we have three pivots covering the entire span of the state.

On the western side, we have Agra as a pivot. Somewhere in the center, we have Kanpur. Then eastern side, we have Gorakhpur. UP also, as you are aware, is known as highway capital of the country. It has a very good infrastructure of highways criss crossing the state and a huge population of nearly 26 crores .

With these three pivots, we believe that leveraging on this infrastructure and potential of the market, we will be able to form maybe two or three clusters going forward and expand our business in UP. It is very striking to note that from literally zero, we will be 1,060 bed capacity in UP by FY '28, in 24 months' time, which is quite remarkable.

Nitin Agarwal: And sir, since you mentioned about the Agra transaction, if you can give us a little bit of color on what is the valuation that you paid for this transaction?

Rajesh Sharma: The total value for this transaction is 245 crores for 360 beds.

Nitin Agarwal: And sir, 360 beds are commission beds?

Rajesh Sharma: Yes, it is already operational.

Nitin Agarwal: And sir, did we share the EBITDA number for the hospital or the last 12 months EBITDA for the hospital?

Rajesh Sharma: Can you just repeat the question once again?

Nitin Agarwal: Sir, the EBITDA number, the annual EBITDA number for this hospital, when we acquired it?

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Rajesh Sharma: No, because currently, they are not operating at a decent occupancy. So even historically also, during our road show, we presented to the investor, we acquired the hospital when they were doing a top line of 50 lakhs, 60 lakhs or 1 crore. Now currently, they are giving us a top line of 21, 22, 23 crores per month.

So when we come on the board, we do put in our effort, we put all the speciality from the day one. So they are also, currently they are doing INR4 crores with EBITDA which is close to 20%.

But once we come on the board, then we are expecting a good growth in the revenue.

Sanjay Sharma: It so happens that in Agra, currently they are operating within 180 beds only, though they have 360 beds, but they are operating within 180. And there also they have about 30% of occupancy.

And the three major specialities are there, cardiology, urology, nephrology being the second one and third one gastroenterology.

When we are opening up, we are opening up with all super speciality, including the neurosciences, including the other specialities and oncosciences. So all put in together when all major super specialities will be open, and we will be operational 360 functional beds, the patient's requirement will be catered to under one roof only. So we definitely see a huge number coming in.

Rajesh Sharma: When we start, we will start with 360

beds.

Nitin Agarwal: Got it, sir.

Nitin Agarwal: And the acquisition price that you mentioned, INR240 crores for a 360 bed is typically higher than our traditional INR35-INR40 lakhs per bed, right?

Rajesh Sharma: Again, it depends on the transaction. As Dr. Sanjay also briefed about this transaction, it is there in the densely populated area. And because we acquired this asset, keeping headroom in mind, even historically, let's say if I talk about our blended capex, we made acquisition of Mohali that was together costed us close to INR275 crores. And the same thing is happening here.

The capex that we are expecting per bed is about 68. But if you see the 2,000 beds that we are adding in the next two years, my blended capex will be INR30-INR35 lakhs only. So here we have done keeping headroom in mind.

Management : You know, looking at Agra, the closest parallel is just so that you understand this is our Mohali acquisition where our capex per bed was similar. So it is the case by assessment, Mohali asset delivered, you know, when we assess its potential, it over exceeded our expectations. It delivered a 36x jump in revenue in 30 months time.

Looking at Agra, we looked at Agra asset in two ways. One is as an isolated case, it is the largest asset in the city of Agra, which has 21 lakh captive population. Influence area should be double of that. So as an individual asset, it has a very high potential for growth in a highly promising

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micro market. And secondly, we looked at it as also an important cog in the wheel for pushing our UP strategy. It gives us a pivot in the western part.

So we have one, as I said earlier, in western part now, one in the centre of the state, one in the eastern part. So as an overarching strategy, it made a lot of sense. In B athinda, our CAPEX per

bed is lower than historical. As Rajeshji just shared, on a blended basis, it still works out for us.

We are at 34 lakh per operating bed number.

Nitin Agarwal: Just a quick last one on this. Do we have a space to add more beds in this hospital at a point in time?

Sanjay Sharma: See, currently our main focus would be that, in 360 bed, we reach the optimum occupancy of about 75 % to 80 %, which is what we call our best foot forward. So there is a long way to go into it. We would have a number of other acquisitions also, as you could say, the pearl necklace between Agra, Gorakhpur and Kanpur, Lucknow, Banaras and other centres would also be there in our mind.

And we will be looking for further acquisitions of Brownfield and Greenfield in between. So currently, we are not envisaging enhancing beyond 360 bed, but we look at ramping up the occupancy to about 75 % to 80 % in the coming three years.

Nitin Agarwal: And the last question from my side, on the business, is there a way for you to characterize our revenues in secondary care, tertiary care, quaternary care? I don't know if this is a very loose definition, but is there a way to say typically how does our revenue really fall in these buckets?

Sanjay Sharma: See, generally speaking, when a human body falls ill, there is nothing primary, secondary or tertiary, to be very honest. The criteria is seen from the services which are being provided by the institution. So we are currently a tertiary, quaternary institutions. All our hospitals are based on that.

Now, if somebody is coming and taking probably a tetanus injection, or a bandaging, that comes into primary. If somebody is having fever or something and taking secondary care, that is also there. If somebody is coming for cardiac surgery, or joint replacement, or you could say organ transplant, that is also there.

So our hospitals are equipped for tertiary, higher tertiary and quaternary care. But per se, different patient mix comes depending upon the requirement.

Nitin Agarwal: Fair enough. Thank you

so much, sir.

Moderator: Thank you very much. Due to time constraints, we'll have to take that as the last question.

Participants whose questions have been left unanswered may reach out to Saloni from the

Adfactors IR team.

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On behalf of Park Medi World Limited, we conclude this conference. Thank you for joining us, ladies and gentlemen. You may now disconnect your lines.

Call: May 2026

May 15 , 202 6

BSE Limited
P.J. Tower,
Dalal Street, Fort,
Mumbai - 400 001
Scrip Code: 544645
National Stock Exchange of India Limited
Exchange Plaza,
Bandra -Kurla Complex, Bandra (E),
Mumbai - 400 051
SYMBOL: PARKHOSPS

Subject: Disclosure under Regulation 30 of the Securities and Exchange Board of India (Listing Obligations and Disclosure Requirements) Regulations, 2015 ("Listing Regulations") - Transcript of Earnings Conference Call

Dear Sir/Madam,

Pursuant to the provisions of Regulation 30 of Listing Regulations, please find enclosed transcript of the Earnings Conference Call held on May 13 , 2026 for Audited Standalone and Consolidated Financial Results for the quarter and year ended March 31 , 202 6.

The transcript is also being disseminated on the Company's website at <https://www.parkhospital.in/>

This is for your information and records.

Thanking you,

For and on behalf of Park Medi World Limited

Name: Abhishek Kapoor
Designation: Company Secretary & Compliance Officer

Encl: A/a

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"Park Medi World Limited
Q4 & 12-Month FY '26 Results Conference Call"
May 13, 2026

MANAGEMENT : DR. ANKIT GUPTA – MANAGING DIRECTOR – PARK
MEDI WORLD LIMITED
DR. SANJAY SHARMA – WHOLE TIME DIRECTOR AND
CHIEF EXECUTIVE OFFICER – PARK MEDI WORLD
LIMITED
MR. RAJESH SHARMA – GROUP CHIEF FINANCIAL
OFFICER – PARK MEDI WORLD LIMITED
MR. SUDESH SHARMA– CHIEF STRATEGY OFFICER
AND GROUP OSD FINANCE – PARK MEDI WORLD
LIMITED

MODERATOR : MR. GANESH NALAWADE – KIRIN ADVISORS

This transcript has been edited for factual errors

Moderator: Ladies and Gentlemen, good day and welcome to the Q4 and 12 -Month FY '26 Results Conference Call of Park Medi World Limited , hosted by Kirin Advisors Private Limited.

This conference call may contain forward -looking statements about the company, which are based on the beliefs, opinions, and expectations of the company as on the date of this call. These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Ganesh Nalawade from Kirin Advisors. Thank you and over to you, sir.

Ganesh Nalawade: Good morning everyone and welcome to the Q4 and 12 -Month FY '26 Earnings Call of Park Medi World Limited. Today we have with us Dr. Ankit Gupta, Managing Director; Dr. Sanjay Sharma, Whole Time Director and CEO ; Mr. Rajesh Sharma, Group Chief Financial Officer; Mr. Sudesh Sharma, Chief Strategy Officer and Group OSD Finance; and the Kirin Advisors team.

We will begin the call with the opening remarks from the management, after which we will have the forum open for the interactive Q&A session. I now hand the conference over to Dr. Ankit Gupta, Managing Director of Park Medi World Limited, for the opening remarks. Thank you and over to you, sir.

Ankit Gupta: Good morning everyone and thank you for joining us today. On behalf of the board and the management team at Park Medi World, we appreciate your time and continued interest in our company. I am very pleased to share with you that FY'26 has been the most defining year in our company's history. An year in which we scaled capacity, delivered our best -ever operating and financial performance, and strengthened the foundation for the next phase of our business growth.

Let me briefly walk you through the key business highlights of the year. In FY '26, we recorded our highest revenue of INR1 ,679 crores and year -on-year growth of 21%. Our EBITDA stood at INR444 crores, which is up 20% on year -on-year basis, and PAT was INR274 crores, translating to an year -on-year growth of 27%. EBITDA and PAT margins were at 26% and 16% respectively.

Our FY '26 year -end ROCE and ROE figures were 18% and 20% respectively, partly moderated by IPO -related equity infusion into the company. The sterling financial performance was backed by equally impressive operational metrics, especially record patient volume and high occupancies.

Talking of growth plans, we added 610 beds during FY '26, signifying over 20% growth in bed capacity in a single year, taking our network from 3,000 beds at the start of year to 3,610 beds as of 31st March 2026. This is the largest single -year capacity addition realized in our history. The capacity addition was realized while maintaining high -quality clinical standards and utmost financial discipline.

To elaborate a bit, the aforesaid capacity addition was in the form of two new units: a 250 -bed facility in Bathinda , which enabled deepening of our presence in the state of Punjab, and a 360 -bed unit in Agra, which marked our entry into the state of Uttar Pradesh. Occupancy ramp -up at both units is trending in line with our expectations.

Separately, we also completed and commissioned our largest -ever Greenfield facility in Panchkula on 10th April 26. Together with the ongoing 150 -bed expansion in Mohali, this will make Park Medi World the largest private healthcare service provider in the Tri-City region. During the year,

we also completed the acquisition of 200 -bedded hospital in Narela, in Delhi, under the provision of IBC, again highlighting our special expertise in identifying and acquiring highly value -accretive assets. We are aiming to commission this unit during Q2 FY '27. On the balance sheet side, we are in an exceptionally strong position. Our gross term loan outstanding was at INR28 crores and we generated INR329 crores of operating cash during the year. This gives us full funding visibility of our stated growth plan to reach 5,460 bed capacity by March 28.

Strategically, Park Medi World is today the largest private hospital chain in Haryana and the Tri-City area, and the second -largest in North India, with a deep cluster -based presence across Delhi and NCR, Haryana, Punjab, Rajasthan, and Uttar Pradesh. We remain a highly capital -efficient, financially strong, doctor -led organization with an unwavering focus on high -quality clinical outcomes and patient care.

So with that, I will hand over to Dr. Sanjay Sharma, our CEO, to walk you through our operational performance in detail. Dr. Sanjay Sharma, over to you, please.

Sanjay Sharma: Thank you, Dr. Ankit, and good morning to the entire august gathering out here. Let me start with our network footprint. As of today, Park Medi World operates 16 hospitals with a combined capacity of 3,960 beds, our largest -ever operational network.

I will be briefly touching on four major points. One is the occupancy of the network. Second is the volume growth in patients. Third would be the case mix. And fourth would be high quality at which we are delivering the healthcare.

Network occupancy for Q4 FY '26 stood at 62.5% compared to 59.4% in Q4 FY '25. The full -year occupancy was 64.1%, an improvement of 244 basis points year -on-year. With regard to patient volumes, Q4 FY '26 IPD volume was 25,329 patients, a growth of 29% year -on-year for the full year. IPD volume stood at 95,525, up 18% year -on-year, and the highest in any year in our history.

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On the OPD side, Q4 FY '26 volume was approximately 1.88 lakh patients, a growth of 13% year-on-year. Full -year OPD volume was close to 7.78 lakh patients, up 22% year -on-year and again the highest ever. Taken together, we served approximately 8.7 lakh patients during financial year 2026, a milestone that reflects the deepening trust which communities have placed across North India into the Park brand.

With regard to case mix, we have been working on high -end procedures and tertiary care, so we continue to see a healthy and a deliberate shift towards higher acuity specialties such as cardiology, oncology, neurology, orthopedics joint replacement, urology , and gastroenterology. Now, on account of that, approximately 56.9% of our revenue has been coming from the high -end specialties, which is an improvement of 316 basis points year -on-year. During the year, we performed over 150 kidney transplants, robotic kidney transplants; approximately 4,700 high -end PTCA procedures approximately 1,200 cardiovascular procedures; and approximately 2,600 robot -assisted knee and hip replacement, reflecting the increasing clinical depth and tertiary care capability of our network.

With regard to the highest quality comparable to the international standards, 15 of our hospitals continue to hold NABH accreditation. Our recently commissioned Panchkula hospital will also be entering the NABH accreditation process shortly. Eight of our Laboratories carry NABL accreditation, and five additional laboratories are being planned for the NABL accreditation process, progressively raising the bar on diagnostics quality across the network.

With that, I will hand over to our CFO to take you through the financial performance in detail.

Mr. Rajesh Sharma, over to you, please.

Rajesh Sharma : Yes, thank you, Dr. Sanjay, and good morning

everyone. As Dr. Ankit and Dr. Sanjay have outlined, FY '26 was our strongest year across every operating metric. And I am pleased to confirm that the financial performance was equally compelling. This was our best -ever quarter and our best -ever full year on every key financial metric.

I'm starting with Q4 FY '26 financial performance. The total revenue of for Q4 FY26 stood at INR460 crores and we registered a growth of 30% year -on-year. Operating EBITDA, excluding other income, came in at INR127 crores, up 44% as compared to last year, the same period, with margin at 28% and expansion of 268 basis points year -on-year. Profit after tax for the quarter was INR77 crores and we registered a growth of 47% year -on-year, with PAT margin at 17%, an expansion of 180 points year -on-year. It was a strong finish to a strong year.

Now I'll talk about the full -year performance. Revenue crossed INR1,679 crores and we registered a growth of 21% year -on-year and our highest -ever annual top line. Operating EBITDA, excluding other income, stood at INR444 crores, up 20% as compared to last year, with an EBITDA margin of 26%. PAT for FY26 came at INR274 crores and here also we registered a growth of 27% as compared to last year and our highest -ever annual profit, with net profit margin of 16% and expansion of 83 basis points year -on-year basis. ROCE and ROE stood at 18% and 20% respectively.

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Now I am turning to the balance sheet. As of 31st March 2026, our gross term debt stood at a standard negligible INR28 crores on a consolidated basis as against INR450 crores in FY '25, which we also plan to repay in the current quarter. Bank balance increased from INR269 crores in FY '25 to INR352 crores, including FDs of INR314 crores in the current financial year.

Debtor days have reduced significantly from 161 days in FY '25 to 129 days in FY '26, this reflecting the improved collection discipline, greater efficiency in government claim processing, and a continued focus on payor mix management. This is a trend we expect to sustain going forward. The net worth of the company has increased from IN R1,052 crores in FY '25 to INR2 ,022 crores in FY '26.

Operating cash flow for FY '26 was INR329 crores. And if I talk about pre -tax, it was INR428 crores as compared to INR215 crores last year, and the pre -tax was INR299 crores in FY '25. Amount spent on capex and the acquisition during the year was INR430 crores, a significant investment cycle that covers the Panchkula Greenfield construction, the acquisition and commissioning of our Bathinda and Agra hospital , equipment upgrade across the network, and the Narela acquisition.

The company already having 1,500 beds under execution, those are expected to be operational by March 2028, which will take our total bed capacity to 5,460 in March 2028. Looking ahead, we remain deeply focused on three things: delivering the best possible clinical outcomes for our patients, scaling our network responsibly and capital efficiently, and creating sustainable long - term value for our shareholders.

Thank you all for your continued support and confidence in Park Medi World. We are now happy to open the floor for questions. Over to the moderator.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Dikshant Gupta from Geojit . Please go ahead.

Dikshant Gupta : Thank you for the opportunity and good morning, sir. First of all, congratulations on the good set of results. Sir, I wanted to know the reason for the margin expansion in Q4. So is it because Q4 seasonally has a better occupancy, or is it because of the ef fect of CGHS, or is it because of the case mix?

Sanjay Sharma: Sorry, Dikshant, you are not clearly audible. Could you just repeat the question, please?

Dikshant Gupta : Hello, am I audible now?

Sanjay Sharm

a: Yes, but it's coming with breaks. Not an issue, would you be kind enough to repeat the question, please?

Dikshant Gupta : Sure, sure. So I wanted to know what exactly was the reason for margin expansion in Q4? So is it because Q4 seasonally has a better occupancy, or is it because of the full effect of CGHS rate hike?

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Rajesh Sharma: Yes, Dikshant, this is mainly because of high occupancy. Because the addition, if I talk about the expansion, the hospital that we added, we added Bathinda, we added Agra in the last year. So Agra we just started from Feb, and Bathinda we started only from May. So this is mainly, that the existing hospital that is running , and we have a better occupancy, that led to higher margin.

Dikshant Gupta : Okay. Okay. And what is the ARPOB for Q4 and for the full year of FY '26?

Rajesh Sharma: The ARPOB, let's say, if I talk about the financial year as compared to FY '25, so FY '25 we were at INR26,200. That grown to INR28,000. And if I talk about quarter basis, so last year, the Q4 we were at INR28,500, that grown to INR29,725.

Dikshant Gupta : Okay. And what is the census bed count out of these 4,000 beds?

Rajesh Sharma: Dikshant, you have to repeat the question once again. The voice is breaking.

Dikshant Gupta : What was the census bed count out of these total capacity of 4,000?

Rajesh Sharma: The census bed, you know, what roughly that is, you know, it is about 82%, 80% census bed if I talk about. And if I say in terms of the number, it is 2,851, and as compared to 2,361 last year.

Dikshant Gupta : Okay, sir. And just one final question from me. So a lot of players in Delhi were facing issues regarding the CGHS rates for cancer drugs. So they were supplied below 30% of MRP . So what impact are you facing from this and suppose government starts supplying at MRP again, so how much would you benefit from that?

Sanjay Sharma : See, Dikshant, since we are largely in the affordable section, we have highly negotiated rates with our vendors. And when this oncology aspect came in, where the rates were to be subsidized for the cancer patients, we had a very prolonged joint meeting wit h all our vendors and we have

agreed to do it with a very low negotiated rates. So we have been able to do it at minimal profits, but we are able to provide the treatment.

Dikshant Gupta : Thanks so much, sir, and all the best.

Sanjay Sharma : Thank you so much.

Moderator: Thank you. Next question is from the line of Vivek from Emkay Global. Please go ahead.

Vivek: Thank you for the opportunity and congratulations on a great set of numbers. So I have a couple of questions with regards to the ARPOB and occupancy. So just wanted to know if you could provide us with the details for ARPOB and occupancy for the, on a cluster-wise basis, for Delhi, Haryana, and Punjab, as these are our main clusters? Additionally, will it be -- so yes, that will be my first question. I'll ask the other questions after this.

Rajesh Sharma: As far as cluster-wise basis is concerned, that we'll provide you once we're done with the call. But on console basis, as I said, you know, that if I talk about the ARPOB, so Q4, if I compare quarter versus quarter to last year, Q4 we were at INR28,423. That grown to INR29,725. But if

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I talk about yearly basis on console, it was INR26,200 and we grown to INR28,000. So we registered a ARPOB growth of 7% as compared to last financial year. As far as cluster-based, data is concerned, we'll provide you post this call.

Sanjay Sharma: Vivek, I'd just like to add in this. ARPOB is not the sole denominator in our profitability and our growth. It is one of the factors which is contributing. And we like to keep the ARPOB within the affordable range because we are largely on the TAM which is about 130 crores of the affordable

e section.

Moderator: Vivek, does that answer your question?

Vivek: Yes, the last part, I couldn't hear the last part. But yes, I guess that more or less answers my question. Moving on to my next question, so just wanted to know from a bookkeeping perspective, what provision have we created for our receivables for FY '26?

Rajesh Sharma: Yes, you know that receivables as far as, because we, our financial, that has to be prepared under IndAS. So whatever the guidelines, and whatever the guidelines which are mentioned in the note, the financial is prepared based on those numbers. And this is, we have created ... (inaudible) we have created provision, so everything is well on track. So if I talk about the number, that is close to INR200 crores.

Vivek : Sorry, could you repeat?

Rajesh Sharma: The number is close to INR200 crores, the provision that we made on 31st March 2026.

Vivek : Okay, got it. Thank you. And just from an understanding perspective, right? So just wanted to understand in case, like for example, how we open a, how we commission a Greenfield hospital or an acquired unit, right? So just wanted to understand what would be the cost associated with a Greenfield hospital when that is commissioned, or an acquired hospital when that is completely integrated into our own entity. And sort of based on that, how do we expect both of those units, achieve break-even?

Rajesh Sharma: Here I can give the answer. You know, let's say if I talk about Greenfield. Greenfield and Brownfield, they are all, you know, they are different ball game. When I say Greenfield, so Greenfield is all, we first have to buy the land, then we have to construct the hospital. I'll give you the recent example of Panchkula. The total capex that we have done is INR125 crores to have 350 beds. Which is more or less aligned with our thought process to have a capex within INR35 lakhs per bed.

And if I talk about the margin, in terms of the recovery, the recovery in terms of recovery, we have seen Greenfield that take about three to four years' time. But when I say break-even, so break-even comes between 12 to 15 months. But Brownfield, historic ally, let's say very recently we started Agra and then we started Bathinda. So Bathinda was a different series that we series call as a blockbuster series, wherein we are able to achieve not EBITDA point, we become EBITDA positive but PAT positive in the first month itself.

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So Greenfield historically we have seen that take about four to six months' time to get the break-even. But recovery, we get the anywhere between two and a half to three years' time.

Vivek : Got it. So my question was more with regards to understanding when the hospital is commissioned, what are the costs or what are the fixed costs that we incur, in the initial phase

especially, just to understand from a margin perspective as to how much we are burning when the hospital is commissioned.

Rajesh Sharma: That's the reason I'm saying. Let's say historically for a Brownfield, as I said, because we are able to make -- we become EBITDA or rather PAT positive in the year, in the first month itself. So there is no burning. But yes, Greenfield we have to bear for 12 to 15 months, initially because when we start, we start full -fledged. So we are not, we never start in a pie ce-meal. So we have a salary cost, I'm talking about the recurring cost that we have, and then we, you know, rest is all variable cost.

So initially, let's say if I talk about, you know, the Greenfield, the first year we normally have we more or less touch EBITDA positive but not PAT positive in year one. But historically we have seen we become EBITDA positive in the year one itself. So th ere is no point of burning the money.

Sudesh Sharma : More specifically, Vivek, more specifically, to answer your question, you see our total capex incurred or th

e sales consideration that we have state for our acquisition so far has been in the proximity of about INR1 ,000 crores thereabout. And the contribution of acquired assets in the revenue, I am talking about FY '26 numbers, is about 61%. That's also about 1 ,100 thereabout. So broadly on a ballpark basis, you can say that the investment we typically on sigma basis deploy ed into acquisition was recovered in terms of revenue realized from these acquisitions in roughly 11 months' time. And the EBITDA break -even has been on a weighted average basis between three to three and a half years' time.

Vivek: Got it. Got it. Thank you and all the best.

Moderator: Thank you. Next question is from the line of Anshul from Emkay Global. Please go ahead.

Anshul: Hi, thank you for the opportunity. Hope I'm audible. First question I had was on the CGHS benefits. While you've put a number in the presentation, I just wanted to get your perspective around what could be the absolute CGHS benefit that we would see trickl ing down in our FY '27 numbers ?

Sanjay Sharma : See, the CGHS had come out with an enhanced hike, about 12% to 15% all across. And Park ends up being the biggest beneficiary of it. But we would also be upgrading a lot of our equipments and high -end procedures will be brought into picture. So that will a dd to the capex point of view. So we are for nine months coming in FY '27, we see an appreciation of about 7 %, 7.5% .

Anshul: So this would be to the revenue?

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Sanjay Sharma : Yes, this would largely be to the revenue.

Sudesh Sharma: Anshul, how CGHS rate hike will unfold in terms of implication to our numbers and business will be as follows. You know one thing that we unequivocally know is that Park is the biggest beneficiary of this very generous rate hike we have seen in the CGHS ra te list. We believe looking at our payor mix on the total revenue, the net impact will be on a conservative basis maybe in the proximity of 5% to 6%.

One very unique feature of Park's business is that, Park's business model is that it is very operationally efficient end -to-end, which means that through its intuitive way of working, internal working, it generates the resources that enable the business to continue the state -of-the-art modern relevant to the current time, allows us the scope to induct new technology as well as expand.

So we believe that this additional resources that this business will generate, if you think about it as a part of business model, which will enable us to continue deploying capital in upgradation of technology and expanding going forward. Had this not been there, this would have been funded by some other form of capital, equity or de bt. So all I'm saying is that while margins, EBITDA and PAT are preserved, the additional resources we generate allow us to stay capital efficient while constantly upgrading our technological edge. I hope this answers your question.

Anshul: It does. I was just trying to categorize margin expansion or sort of keeping margins intact from the benefits of CGHS, especially, I had an allied question on this. We would have new units being commissioned in Punjab and Agra in the current year. Would that sort of drag margins, which sort of be pulled back because of this CGHS rate hike? How should we think about margins in FY27? Sorry, I mentioned the wrong units. How should we think about margins in FY'27 because we have Greenfield additions in Panchkula and Rohtak?

Sanjay Sharma : See, Anshul, Rohtak will be coming in January 2028, 250 beds. Panchkula we have already commissioned on 10th of April 2026. So that will be definitely delivering and it is already one of the, you could say, top -notch hospitals in that area. And in the Tri -City, we'll with the addition of 150 beds in Mohali, we'll be the largest healthcare provider in the Tri -City with 850 beds.

Now why we ha

ve done this is this because there is a lot of patients which come from North India, J&K, Himachal, Uttarakhand, upper side of Uttar Pradesh and all that, who travel to Delhi for treatment, which we will be catering to. So that is the advantage which will be provided to the patients that they will have more economical treatment in the Tri -City, which will be cost - effective also, which will be beneficial to the patient also, and which will drive our margins.

Anshul: That's clear, sir. I just wanted -- so are we saying that Panchkula facility will not be a drag on our margins in FY '27 despite the Greenfield nature and despite the size of the facility?

Rajesh Sharma: You know, if I talk about the Bathinda, so Bathinda, as I said, because we added in July 2025, that is already adding into our numbers. So we are doing extremely well in Bathinda. So rather we have started operating on an occupancy of 65% to 70%. So there we are, that will add significant in terms of our top line and also on the bottom line.

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And if I talk about Agra, I think specifically talking about these two units, so if I talk about Agra, so Agra we started from Feb 2026. It is, that it's already adding close to INR5 crores in our top line per month. So current year, we are expecting a top line, it will add about INR90 crores in our top line and it will become EBITDA positive. So next current year, Agra will add into the revenue, but not much on the EBITDA margin.

Anshul: Got it. This is helpful. Also, if I could ask a question on the sustenance of the payor mix. While CFO sir alluded that you guys think that this is sustainable, do we have any target in mind of sort of keeping scheme mix at an X percentage going forward?

Sanjay Sharma : Yes, Anshul, see, we have always had a vision to provide relatively affordable healthcare at the highest quality. And we are not targeting the payor mix per se since the beginning of our endeavor. And going forward also, we would like to be in the same segment. Now largely that segment is some or the other way provided by the government insurance. But our payor mix on its own, because a lot of affordable sections in the self -pay and private insurance are also having limitations to go to the premium brands .

So they are gradually shifting for treatment to Park. And that percentage is regularly increasing. So currently we are looking at by the end of this financial year, hopefully that percentage will be 65 % coming from government insurance, it should be of 70% coming from government insurance, which will be largely central government, and about 30% coming from the private insurance.

Sudesh Sharma: Anshul, the most important point to notice here is that , in line with often stated policy of Park, we are not still cherry -picking patients. The patients continue to be very democratic and organic. As the patients come exercising a choice to use Park's services, our intake will reflect whatever mix that is. As Dr. Sanjay highlighted, it just so happens because of increasing preference being exercised by patients belonging to even the cash TPA segment, in terms of value proposition that Park offers, this mix is changing.

We see that on ground. So we are, we are nearly at 80 -20, if we should call this government insurance schemes and cash TPA segment. This is the patient mix which gets reflected in the revenue mix accordingly. So we believe going forward gradually, but surely, this will change to 75-25 then 70 -30.

And most importantly, I think sometimes the undertone in this question is concerns people have concerning receivable cycle. So I would like to highlight that what dramatic positive outcome we have had in this financial year has been a very sharp reduction in our working capital cycle, receivable cycle, despite nearly 21% growth in realisable revenue. I think this is quite a staggering

achievement.

Our collections ballpark basis has been nearly 100%, which should dispel any concern concerning government insurance schemes portion of the revenue, accounting for delay in receivables. Our collections have been extremely robust and strong, but that said, this gradual shift in mix is there. We believe this will stabilize in 70 -30 split band, in due course of time, maybe 12 months, 18 months thereabouts.

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Rajesh Sharma: And if I see historically data, you know, FY25, we were at 89%. Now we more or less came

down to 80%. It's already reflecting in our numbers. And going forward, as Dr. Sanjay and Sudesh ji mentioned that, you know, we are well on track to reduce further.

Anshul: Excellent, excellent. Thank you for the detailed answer, that was clear. Just one last question if I can squeeze in...

Moderator: I'm sorry to interrupt, Anshul. Please rejoin the queue for more questions. Thank you. We will take our next question from the line of Debanjan Bhakta from Universal Sampo General Insurance. Please go ahead.

Debanjan Bhakta: Hi, thanks for the opportunity. Am I audible?

Rajesh Sharma : Yes, please. Yes.

Debanjan Bhakta: So are there any plans to improve the oncology mix and are we doing radiation oncology?

Sanjay Sharma : Hi Debanjan. Yes, we are increasing our oncology footprint everywhere. In fact, currently we have it in three locations. Ambala we have 200 beds which will be coming up for an extension where it will be dedicated to the oncology unit largely. And we have oncology as in medical oncology and surgical oncology in all our 16 hospitals. But the radiation oncology along with it will be shared between those 16 hospitals because we have it currently in three hospitals. But gradually as the need arises, we'll be strengthening this vertical more and more.

Debanjan Bhakta: Okay. And I have two more questions. One is what is the ALOS for chemotherapy patients? Hello?

Sanjay Sharma : Yes, could you repeat the question please?

Debanjan Bhakta: What is the ALOS for chemotherapy patients?

Sanjay Sharma : See, chemotherapy patient, chemotherapy is basically a day-care procedure. And patient come out there and take chemotherapy. So is with the radiation also. It is only the treatment planning and if there is a surgical aspect of the oncology, which takes time. So it's a continuous process based on the cycles patient come and receive the chemo and if required radiation. Surgical procedures are generally one off or at the maximum twice undertaken based on the spread of the cancer and how localized it is and if it can be surgically extracted.

Rajesh Sharma: And I wish to add one more point, regarding you know, onco contribution to our revenue, if I compare, if I give you the quarterly number, so it is contributing more or less 8% of our revenue as compared to 6% in a same period last year, if I talk about Q4 FY25 versus Q4 FY26. And if I talk about annual, it is 5% of the top line last year, that grown to close to 6.5%. So that means, onco is contributing more to our top line.

Debanjan Bhakta: And I have a last question, that is, are we see...

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Moderator: I'm sorry to interrupt, Debanjan. Please rejoin the queue for more questions. Thank you. Next question is from the line of Sagar Tanna from Alchemie Ventures. Please go ahead.

Sagar Tanna: Hi sir. Just a clarification on the CGHS rate revision. Has anything flowed through in FY26?

Sanjay Sharma : Hi Sagar. See, CGHS has number of line items from 4,500 to 5,000. CGHS is the apex body which floats the rate. And then there are number of other central government agencies which take that rate. And they have to incorporate and there are various permutations and combination. Yes, some flowing of that rate has come in, but actual impact probably will be seen after the

Q1

FY'27, to be honest, in the complete format.

Sagar Tanna: Got it. My second, my question is on the strategy side. Now we have entered the state of UP, three hospitals we have announced, right, which will be commissioned by March '28, FY '28.

Any thoughts in terms of how we will would like to expand further into UP?

Sudesh Sharma: Sagar, UP has been in our crosshairs for quite some time. You will have noticed whichever state we have been present in, we have always endeavoured to realize a critical size. Because that's whenever synergies come into play and benefit our business. So in the state of UP, what is important to note is from literally zero till two months back after Agra acquisition, which got commissioned middle of February month this calendar year, by FY '28, we will have nearly 1,060 bed capacity in a state which has a population of nearly 26-27 crores.

So imagine, first of all, setting the agenda why we like, why we really want to expand in UP.

That kind of TAM, equivalent to a country, large country, in a population profile which especially benefit from Park's value proposition of relatively affordable high-quality healthcare services.

Second, UP is rightfully called the highway capital of the country. Excellent road infrastructure, which allows a company like Park to deploy cluster format of expansion by seeking the locations in close proximity to highways, a prominent route.

And third is the beginning we have made and the plan we have outlined. If you can visualize for a moment the map of UP, after launch of Agra, which is on the western side, becomes our pivot for expanding in that part of the state, we will have an asset shortly in the middle of the state,

and ideally according to our plan to expand in the location of Kanpur, then Gorakhpur on the eastern side. So technically, we will be spanning the whole state. And keeping into consideration size of UP, we will have opportunity to create multiple clusters around these three pivots. But I think that's one way to look at our UP strategy, the more important way is to assess the high impact we endeavor to make in the state. Agra, for example, has an immediate micro market of nearly 21 lakh population. Gorakhpur is said to be having a micro market of 12 lakh. Kanpur is about 20 -21 lakh. So that's about 50 -55 lakh immediate market.

And if we should expand the circle of influence, normally you multiply the immediate micro market numbers by two, that's about 1 crore immediate by means of the points we talked about,

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by means of these assets spanning the state in as optimal a manner as possible. We will be providing access to our high -quality and relatively affordable healthcare services to nearly a crore of population.

That is something staggering. And all of this unfolding in front of your eyes, starting from Agra commissioning two months back to completing this agenda by end of financial year 2028. So this is how we look at UP as we speak.

Sagar Tanna: Got it. Thank you so much and all the best, sir.

Moderator: Thank you. Next question is from the line of Anant Sarda from Chhattisgarh Investment Limited. Please go ahead.

Jhalak : Yes, I'm Jhalak . So my first question is, how is the company funding the new acquisition? Like, it's from the fundraise only or you're taking more debt? Is there any plan to take debt in future also?

Rajesh Sharma: You know, as far as future expansion is concerned, that we are adding close to 1,500 beds by March 2028. But requirement is not that, as I said, historically also we are maintaining lowest capex in the industry. Going forward also, we have the same plan. The total capex for next two years, two years is close to INR500 crores.

And if I talk about our current financial position today, so we are sitting on FDs of INR314 crores, we have close to INR100 crores in our bank account. So we ca

not foresee every year

we are generating a OCF close to INR30 crores. So we may go for a small debt, but not the major one, because we have enough money with us.

Jhalak : Okay. And my next question is, what does it constitute in your other non -current financial asset?

Rajesh Sharma: Non-current financial asset.

Moderator: I'm sorry, sir. We are not able to hear you. If you are speaking.

Rajesh Sharma: Non-current, you know, these are the FDs which is more than 12 months.

Jhalak : Okay. Can you repeat the amount?

Rajesh Sharma: Can you hear me?

Jhalak : Yes, yes. Now I can hear you.

Rajesh Sharma: If I talk about as of 31st March 2026, we have FDs which is close to INR135 crores, those are more than one year. So that falls under financial asset, non -current financial asset.

Jhalak : Okay. Thank you so much, sir.

Moderator: Thank you. Next question is from the line of Akshay Thakur from Helios Capital. Please go ahead.

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Akshay Thakur: Hi sir, thanks for the opportunity and congratulations on the good set of numbers. Sir, I wanted to understand your acquisition strategy. So basically, the strategy for the future is to do a Brownfield expansion primarily and there are acquisitions planned . So I wanted to understand how do you choose these assets in terms of metrics you track for the asset to be eligible to be, included in the Park hospital chain?

Do you track, look at the metric in terms of financial numbers or is it more on the side of payor mix or specialty mix? I was just wanting to understand your broad strategy on that.

Sanjay Sharma : Hi Akshay. See, out of 16 hospitals, six hospitals are Greenfield and 10 are acquired assets. Now, we had about 50 -60 propositions out of which we had been very diligent in picking up these 10 assets. And the factors which made us go for it largely is the strategic location of the units, the domain strength of the region, the medical facilities available and not available in those

areas, and the headroom for expandability.

So these are the four aspects. And the fifth is largely that the distress assets which they were, they were bleeding profusely, and fast payment after the due diligence has been done, also made the deep discounting possible. So these are the five factors which we generally go for. With regard to since they're already bleeding, so the EV -EBITDA multiple does not matter and the financial aspect does not matter.

And as we had discussed before, we do not go for the payor mix. We are largely going in the affordable section, so keeping it relatively affordable, whatever the payor mix comes through, we cater to that. So we do not cherry-pick the patients in that perspective.

Akshay Thakur: Thank you, sir. That was helpful. One more question, so if we look at our ARPOB today for FY'26, it was around INR24,500, now it's around INR29,000. That gives like growth of 5% to 6%. So primarily, I assume this growth, ARPOB growth would be coming from improvement in your mix, which you have mentioned. Because the CGHS rates and everything, hikes happen once in two years.

So how do you address this challenge, so your cost, inflation hike of doctor's salary and consumables and everything would be somewhere around range of 8% or more. So how do you address this challenge, like, of increasing your ARPOB in future for the coming year?

Sanjay Sharma: See, Akshay, as we mentioned before, ARPOB is not the single determinant for the profitability. ARPOB is one of the factors which contributes to the profitability. Since our vision has been relative affordability and high quality, a lot of factors which contribute into it. One is this that our capex is the lowest. We are on a console basis till financial year 2028, with the capacity expansion to 5,460 beds, we would be looking at a capex of about INR34 lakhs. Our nearest competitor would be more than

double of this capex. So this is one advantage which we create. Also, keeping the quality and the NABH compliance, we accommodate more beds in lesser space. 30% beds are dedicated for critical care and 40% is allocated for general ward. Am I audible?

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Akshay Thakur: Yes, sir.

Sanjay Sharma: So 70% beds are allocated in a lesser space, which also gives us a huge advantage. The other factors are extremely well-placed vendor management, which we have an excellent relationship for past 29 years, and which we do not keep any debtors more than a month. Whatever the debt of the vendor have given us within this month is cleared by the next month 10th to 15th of next month. So that by itself gives us a huge discounting in that.

We have a construction unit which is largely dedicated to us. So any renovations or any constructions in the Brownfield or the Greenfield respectively is done at a very fast pace. So there are a number of contributors besides operational efficiencies and very efficient SOPs which have been on running the hospitals.

Sudesh Sharma: In short, don't the expenses benchmark, please don't benchmark them to ARPOB alone. We are very, very disciplined in terms of our financial numbers, ratios, highly disciplined. So please see expenses as a percentage of revenue. So revenue has lot of contributors, ARPOB is one of them. I think understanding our business model, seeing expenses in relation to ARPOB alone might lead to little, little off-the-market interpretations.

So a humble request, a suggestion, please see expenses as a percentage in terms of how they relate to the revenue, profitability numbers, not ARPOB alone. For example, yes, maybe I will just throw an important point, maybe I'll take a few seconds more to further throw light on this point. What goes into revenue, you see is that we might have an asset, in fact, let me just quote a live example, one of our Gurgaon assets might have 275 beds with an ARPOB of INR32,000, so which gives the revenue of INR32,000 spread over 275 beds.

So what we are having revenue in this unit is 275 beds doing an ARPOB of INR32,000. So if, you know, if you think that one could run this location with 100 beds and an ARPOB of INR80,000, the revenue will be still the same. So what we're saying is, the right way to benchmark expenses is to revenue, not to ARPOB. So technically 100 beds with INR75,000, INR80,000 revenue is nearly the same thing as INR32,000 revenue across 275 beds. Therefore, re-emphasize, expenses about which we are very disciplined as a percentage are marked to revenue, not to ARPOB alone.

Akshay Thakur: Thank you, sir. That was helpful. Can I squeeze in one more question if I could...

Moderator: I'm sorry to interrupt, Akshay. Please rejoin the queue for more questions. Thank you. Next question is from the line of Shubham Padiyar from Chhattisgarh Investments Limited. Please go ahead.

Shubham Padiyar: Yes, hi. Good morning. So I just wanted to understand what was the reason for our drop in

standalone numbers quarter -on-quarter? I know we should look at consolidated, but just to have a better sense of our numbers. So what was the reasons?

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Rajesh Sharma: Wait, you are comparing in terms of what? Our last quarter, so we have a growth. We were at INR354 crores in terms of revenue and that grown to INR460 crores.

Shubham Padiyar: I'm talking about standalone quarter -on-quarter. So let's say Q3 versus Q4 of FY '26.

Rajesh Sharma: You talk about Medi World alone?

Shubham Padiyar: Yes.

Rajesh Sharma: Yes, because, that if you see the growth, that we made, that, if I talk about Q4 versus Q3, we were under -- I'll give background in terms of the Bathinda hospital. When we acquired that hospital, that was under O&M and that was run by Park Medi World. But in Jan, what

we have

done, on 5th of Jan, we made the acquisition of that hospital that become a independent subsidiary.

So as far as in console, there is no impact in terms of the top line, but yes, the revenue which was coming in Park Medi World, now that is going in that Bathinda company. That is the only reason.

Shubham Padiyar: Understood. And also one more question on our disallowance provision of government revenue. I'm not able to see that figure in our P&L for this quarter.

Rajesh Sharma: So disallowance remain, that as a -- because we are very, very particular about our disallowance and we have a rigorous process to reduce as much as possible, but yes, we are remain at 9% disallowance, even in Q4.

Shubham Padiyar: All right. Got it. Thank you, thank you so much and best of luck.

Rajesh Sharma: Thank you.

Moderator: Thank you. Next question is from the line of Chetan Shah from Jee t Capital

Chetan Shah: Yes, thanks. So just thank you for a very detailed answer to all the questions. Just one small question in terms of the long -term growth strategy. If we see our current cluster of capacity and concentration which is into three or four states, if one wants to pencil the opportunity for next three to five year time horizon .

Do we still keep following into this specific states and region or we are also looking at expanding beyond these and continue to focus on a similar hub -and-spoke cluster expansion model? Just to understand the long -term view of the management and how they are thinking ? Thank you and congratulation once again, sir.

Sanjay Sharma : Thank you so much, Chetan. See, our approach in growth has been very strategic. Currently we are present in five states. We to some extent consolidated ourselves a little in Delhi, Haryana, somewhat in Rajasthan and Punjab where we are also further endeavoring to consolidate.

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But besides that we entered UP. Now what we are -- the strategy is this, that each hospital which we bring is about 40 -50 kilometers to the other hospital, so that it adds synergy in -- you could say, multiple ways with regard to the human resources and with regard to the high -end equipment. So that creates the synergy.

We are not in the hub -and-spoke model at all. Each unit is independent on itself and caters to the all super specialties. But certain top -end, high -end like robotics or you could say major minimal excess surgery or you could say joint replacements or kidney transplants or radiation . These are the areas which are shared between two to three hospitals. And that is how the synergy is brought in and the capex and the opex is kept within control.

Chetan Shah: Understood. Got it. So sir, is there one of the reasons why our, if one looks at on a blended basis, capex per bed or opex per bed is one of the reasonable in the industry and that's why in spite of comparable ARPOB our margin per bed is relatively better and so does our payback period.

That's the right understanding, correct?

Sanjay Sharma : Yes, you hit the nail on the head. In fact, I would just like to add that currently we are present in 16 districts of North India. There are about 174 districts. We would like to consolidate in all those areas initially and then expand into South , East West. But if any good opportunity comes in and it helps us to go in a strategic manner, we are open to it.

Chetan Shah: Got it, sir. Thank you so much and wish you all the best.

Sanjay Sharma : Thank you, sir.

Moderator: Thank you. Next question is from the line of Ashutosh Adsare from USGI. Please go ahead.

Ashutosh Adsare: Yes, thank you. Thank you for giving me this opportunity. Good morning everyone. I just wanted to understand, we have 1,500 bed additions slated for FY '28. So does it mean that we are curbing our growth given the opportunity that our OCF is at around INR350 -odd crores? Our debt level

is around INR28 crores only which will be paid off? So do we think we are curbing our growth

for next two years or should we be on a higher growth trajectory given our financials?

Sanjay Sharma: Ashutosh, we are not curbing our growth at all. The statement which we are giving in the future projections are largely the deals which are more or less cast in stone. We are looking at 850-bed expansion in this coming financial year, out of which 350-bed have already come in Panchkula, 200 would be coming in Narela, Delhi, and 300 would be coming in Kanpur.

FY'28 we will be adding 1,000 beds, which will be coming 400 from Gorakhpur, 200 would be coming from Ambala extension in the oncology department, 150 would be coming from Mohali expansion, and 250 Greenfield will be coming from Rohtak.

But if any other project or any other unit comes which is quite lucrative and as per our model, we would be more than open and willing to go ahead from that. The point which Mr. Rajesh emphasized is this, that since we are a cash-rich company, generating about INR330 -INR40

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crores in cash, and plus we have money in FD also in current account, the debt aspect is not that major currently.

And going forward also, if a major, you could say, big unit or a chain of units which we find lucrative, then we'll definitely go for debt also. So there is nothing curbing our expansion.

Rajesh Sharma: Yes, Ashutosh, one point here is to add. When we did our listed on 17th of December, there was no way a discussion was. But we ended up closing that deal from 1st of Feb. So this all happen on a period of, you can say about 30 -35 days. So we are there in the market, we are aggressively looking for the option. And if we get something, we are more than happy to close, and funds will never become a hindrance.

Ashutosh Adsare: Okay. And on the receivables front because due to better collection efficiency, we are driving this 129 days to be there for FY '26. So going forward, is there any levers to bring it down below 100, so that it will not have impact our working capital management?

Sudesh Sharma: Ashutosh, just keep to repeat. Your -- this question didn't come out clearly.

Rajesh Sharma: Yes, I'll answer this question. You know that as far as debtors is about, if I talk about the total debtors, 92% relate to Central Government. And when I say Central Government, because they have a well-defined process and it is divided into two parts. One, there is a bill settlement and there is a payment process.

The bill because when, when the patient comes and when the patient get discharged, we have to upload that bill within seven days from the -- to the third-party, service provider, which is hired by the Central Government or the respective panel. And thereafter, they have a process to clear the bill within a period of three months.

But here, lot of pressure is coming on the government to minimize this period. So this whatever has happened over 30 -32 days, the advantage that we got in terms of debtor days, that is only because push which is coming from the government to streamline this process.

Because earlier it used to take three months they take to normally clear the bill and 45 days' time to make the payment. But now the 3 month period government is pushing to come down to two months. The process is going on. We have seen that result, but how it will turn going forward, it is too early to say, but we are expecting that it will be within this range of four months.

So I cannot foresee that this happening, less than 100 days in next six months or nine months.

But what we are expecting by maybe current year or maybe by next year, the way government is focusing on, they may bring down to less than 100 days.

Ashutosh Adsare: Okay. Thank you, that's it.

Moderator: Thank you. Next question is from the line of Vidhi Dadial from

Raedan Capital. Please go ahead.

Vidhi Dadial: Good morning, sir. Thank you for the opportunity and congratulations on a good set of numbers.

Sir, can you just help us with the break-up of incremental bed expansions between FY '27 and

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FY'28? And if you could also throw some light on the visibility beyond FY '28 for bed expansion, if the company has worked out any kind of blueprint, rough blueprint, for bed expansion beyond FY '28? Thank you.

Rajesh Sharma: I'll answer the first question. You know that the capex that we plan going forward, as we are adding 1,850 beds, whatever the under execution, of which we have already added 350 bed in Panchkula. So there we invested about INR125 crores. That is already done.

Now we are coming up one in Delhi, that is Narela, where we added, 200 bed. The total capex we plan to do is about INR80 crores, wherein INR55 crores we already spent. So there we need to add, we need to spend about INR25 crores. And Kanpur we are -- we have to invest about INR30 crores.

So for the current year, we plan do the capex close to INR55 crores. And for the next year, so next year we are adding 1,000 bed, and together -- that, together we are expecting close to INR250 crores of capex in FY28.

So that's, as far as our plan in terms of the capex. It is well within the resources that we have.

And as I said, because we are not expecting any further debt raised or maybe a major debt raised, whatever we have in hand today.

Sanjay Sharma : But in future aspect which you have asked, between FY '28 to FY'33, we are hoping that we'll probably be able to double this strength of 5,460 to nearly 10,000 plus.

Sudesh Sharma: I think what needs to be understood is that there is a 1,500 -bed capacity augmentation that is outlined for the period from now till FY '28, which is crystal clear. These assets are under execution.

Vidhi Dadial: Okay, sir. That's really helpful. Thank you so much.

Moderator: Thank you. Next question is from the line of Debanjan Bhakta from Universal Sompo General Insurance. Please go ahead.

Debanjan Bhakta: Yes, thank you for the follow-up. Are we seeing any sort of demand-side problem as we are expanding in the Tier-2 cities?

Sanjay Sharma : No, in fact, we have seen surge in demand with regard to all aspects. I give you an example. As we are expanding into Tier-2, Tier-3 cities, the requirement of high-end, high-quality at a relatively affordable cost treatment has always been there.

And in fact, patients were traveling far and out to the metros and sometimes that valuable golden hour in critical illness was being lost and lot of people were not able to survive. So this is an advantage which we have created by operating in Tier-2, Tier-3 cities.

Also with regard to the top-end doctors and clinicians, since they have all the facilities now available in their hometown, lot of people who were forced to actually go and work in far-off

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stations away from their family and hometown, they have been coming over. So even in Tier-2 and Tier-3 city, we have seen a large volume of doctors wanting to come out there.

The only thing was that they did not have a level playing field to execute their skill sets and the academic sets because those structures were not there. But ever since Park has come in Tier-2, Tier-3 cities, they are more than happy to work in these areas and satisfy their skill sets.

Debanjan Bhakta: Okay. Thank you.

Moderator: Thank you. Next question is from the line of Parshwa Shah from Mehta Equity . Please go ahead.

Parshwa Shah: Thank you for the opportunity, sir. I want to ask about capex. So we are doing well in the capex. I want to understand about new capex, whether Greenfield or Brownfield capex. So what are the biggest execution mistakes that hospital

operators make while entering in new geographies?

Sanjay Sharma : Sorry Mr. Shah, would you be kind enough to repeat the question because the audibility at the moment is low.

Parshwa Shah: I want to ask about what are the biggest execution mistakes hospitals have made when they entering the new geographies?

Sanjay Sharma : Okay, Mr. Shah. See, generally what happens is this, that why we have had a strategic approach in expansion is that one hospital should bring synergy to the other hospital which is in the vicinity of about 50 kilometers or so. But generally what other brands have been doing is this that they've been expanding into remote areas and at times the replication of the values, the regulations, the way they've been functioning, their first or second hospital, they've not been able to replicate.

Few good things which we do, which we have been able to emulate all our hospitals in the same way and replicate it is largely because second-line management for the new hospitals which we are taking is already in current training in our current operational hospitals.

So there are top management which is being prepared in the current hospitals, which are aware about it and they've been training for about three to six months and they get transposed to the new unit wherever we are bringing in. And the new unit is generally not very far from the other

units .

If you take it in very far -off domain or in a domain which has got a different positives and negatives, a lot of brands find it difficult to replicate those aspects and taking the management afresh, the values cannot be replicated.

Parshwa Shah: Right, right. Got it. Thank you, sir, and all the best for next quarters.

Sanjay Sharma : Thank you so much.

Moderator: Thank you. Next question is from the line of Akshay Thakur from Helios Capital. Please go ahead.

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Akshay Thakur: Thanks for staying long on the call. I really appreciate that. Sir, one question on, when we acquire an asset, there would be doctors which would be already in the unit. And we would like to also to bring in our doctors and bring in the operational efficiency. So can you just explain or just throw what are the key metrics or the key things which you work on primarily? I am assuming that would be -- one of those would be the doctors to bring in the efficiency and how is it executed?

Sanjay Sharma : See, Akshay, the acquisitions which we have done in past historically suggest that they've been distressed units and they've been bleeding profusely. And entirely it would not be promoter's fault also. It would also be to some extent the fault of staff, the management which had not been performing.

So generally it happens that when we take over a unit, whatever staff is efficient and is able to deliver as per our norms and as per our vision, we retain, but the other staff needs to be changed.

So this is the policy which we go ahead with. That if the staff is underperforming or if there is management which is under performing, we go for a change.

Akshay Thakur: Okay, that was helpful. Any investments on the tech front to bring in the efficiency?

Sanjay Sharma : See, most of our doctors, the clinician which we have are from very good reputable government colleges with at least five to seven years minimum experience with hands -on experience with the latest technology like robotics, the medical advancements, lasers, minimal access surgeries, and so forth and so on.

So these are the doctors which we take. And if any training is required, that is also imparted to them. And whatever top -end equipment is required for the functioning, efficient functioning, and better clinical outcomes, that is part of the capex which we ensure that we deploy.

Akshay Thakur: Okay, that was helpful sir. Thank you.

Sanjay Sharma : Thank you.

Moderator: Thank you. Next question is from t

he line of Shubham Padiyar from Chhattisgarh Investments Limited. Please go ahead.

Shubham Padiyar: Yes, hi, thanks for the follow -up. So I just wanted to understand the competitive landscape that we operate in. Park is a sort of a affordable hospital in Northern India. So is it that there's no other player offering similar kind of services at that price and that's why we've been able to capture that vacuum or is it something else?

Sanjay Sharma : See, it will be more from my personal experience which I would be sharing. There have been some players which have been striving to address this TAM in the relatively affordable and trying to provide high quality. But unfortunately the replication or the expansion has not happened to that extent what Park has done.

I don't see any of record if I take there have been some hospitals which tried to enter into it, but the sporadic spurts on far -out locations, they had to increase the capex, they had to go from the

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affordability aspect that has happened. There are multiple factors if you would like to go with this vision of providing relative affordability and high quality.

And still be profitable is to go grow strategically, ensure the acquisitions or the Greenfield are very systematically and very strategically looked upon which does not increase the capex, keeps the expense limited and ensures revenue. So these are some of the factors which has to be seen very diligently.

Besides that the first hospital which we've been running and the subsequent which we've been running, the values, the SOPs, the operational efficiencies, we've been able to replicate because largely the top management, as I mentioned before, have been trained for three to six months in

the current running hospitals.

So there are a lot of units which tried into this, there are some standalone hospitals also, but unfortunately to this scale in this format, in this structured manner, I don't foresee any other hospital with such low capex and delivering such good results with affordable ARPOB and excellent clinical outcomes.

Shubham Padiyar: Understood. Thanks for that. Also do we track doctor attrition rates in our hospitals?

Sanjay Sharma : Yes, we do track the doctor attrition rates. And in the consultant level the attrition rate is one of the lowest because there are multiple factors which are contributing to it. One is this that our selection process is very methodical. And a lot of strengths of the clinician as well as the placement of the clinician based on the requirement is seen.

HR process is very robust in which they help in accommodating the doctor, their families and the education of their children in that region. We also have -- we are one of the excellent paymasters in ensuring that the doctors are paid well. And besides that, since Park by itself is a recognized brand in North India, these doctors are not accountable for volume or revenue unlike other premium brands.

These doctors are accountable only on two major verticals. One is patient and attendant satisfaction and the other is excellent clinical outcomes. Based on that from each unit we have a -- you could say a percentile clocking system in which the points are given to these clinician. And any clinician which clocks more than 75 percentile is personally recognized and rewarded by the promoters in the form of performance bonus. They do not have the pressure of bringing volume or revenue to the hospital. We have our own patient flow. The only thing which they are responsible is to ensure that all the patients and their attendants are satisfied and they get the best clinical outcome.

Shubham Padiyar: Understood. Thank you, thank you for that answer.

Sanjay Sharma : Thank you.

Moderator: Thank you. Ladies and gentlemen, we will take that as the last question for today. I would now like to hand over the conference

to Mr. Ganesh Nalawade for closing comments.

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Ganesh Nalawade: Thank you everyone for joining the conference call of Park Medi World Limited. If you have any further queries, you can write us at research@kirinadvisors.com. Once again, thank you everyone for joining the conference.

Moderator: Thank you very much. On behalf of Kirin Advisors Private Limited, that concludes this conference. Thank you all for joining us today and you may now disconnect your lines.